BY ORDER OF THE SECRETARY OF THE AIR FORCE

AIR FORCE MANUAL 10-3509

17 AUGUST 2026

Operations

AFSPECWAR MEDICAL OPERATIONS

COMPLIANCE WITH THIS PUBLICATION IS MANDATORY

ACCESSIBILITY: This publication is available for downloading or ordering on the e-Publishing website at www.e-Publishing.af.mil

RELEASABILITY: There are no releasability restrictions on this publication

---

OPR: AF/A34S | Certified by: AF/A34S

Supersedes: AFMAN 10-3509, 22 December 2020 | Pages: 39

This manual implements Department of the Air Force Policy Directive (DAFPD) 10-35, *Air Force Special Warfare*. It establishes the Air Force Special Warfare (AFSPECWAR) medical operations and human performance optimization (HPO) programs. This publication applies to all civilian employees and uniformed members of the Regular Air Force, the Air Force Reserve, the Air National Guard (ANG), and those who are contractually obligated to comply with Department of the Air Force (DAF) publications. This publication does not apply to the United States Space Force. For the purposes of this manual, all references to major commands (MAJCOMs) are intended to also reference or include the ANG. Ensure all records generated as a result of processes prescribed in this publication adhere to Air Force Instruction (AFI) 33-322, *Records Management and Information Governance Program*, and are disposed in accordance with (IAW) the Air Force Records Disposition Schedule, which is located in the Air Force Records Information Management System. Refer recommended changes and questions about this publication to the office of primary responsibility (OPR) using the DAF Form 847, *Recommendation for Change of Product*; route DAF Forms 847 from the field through the appropriate functional chain of command. This manual may be supplemented at any level, but all supplements must be routed to the OPR of this publication via email to **AF.A34S.Workflow@us.af.mil** for coordination prior to certification and approval. The authorities to waive wing/unit level requirements in this manual are identified with a tier ("T-0, T-1, T-2, and T-3") number following the compliance statement. Submit requests for waivers through the chain of command to the appropriate tier waiver approval authority, or alternately, to the requestor's commander for non-tiered compliance items. See Department of the Air Force Manual (DAFMAN) 90-161, *Publishing Processes and Procedures*, for a description of the authorities associated with each tier. Approved T-2 and T-3 waivers will be sent to AF/A34S within three business days for tracking and process improvement. The use of the name or mark of

any specific manufacturer, commercial product, commodity, or service in this manual does not imply endorsement by the DAF.

SUMMARY OF CHANGES

This manual has been significantly revised and must be reviewed in its entirety. Significant changes include expanding on medical operations to include flight medicine and HPO. Finally, the manual has been updated to reflect changes to the United States Air Force (USAF), Defense Health Agency (DHA), Air Force Medical Command (AFMEDCOM), and the AFSPECWAR community since the previous release.

Chapter 1—INTRODUCTION **4**

1.1. Overview. 4

1.2. AFSPECWAR Medical Operations. 4

Chapter 2—ROLES AND RESPONSIBILITIES **5**

2.1. Air Force Surgeon General (AF/SG). 5

2.2. Air Force Special Warfare Division (AF/A34S). 5

2.3. Air Force Special Warfare Medical Director (AF/A34SG). 6

2.4. Air Force Special Warfare Human Performance Optimization Director (HPO Director). 7

2.5. Air Force Pararescue Medical Director (PJ MD). 7

2.6. Air Force Pararescue Medical Programs Manager (PJ MPM). 8

2.7. Air Force Pararescue Medical Materiel Manager (PJ MMM). 9

2.8. MAJCOM Headquarters Staff. 9

2.9. Major Command Surgeon (MAJCOM/SG). 10

2.10. Unit Squadron Commanders (SQ/CC). 11

2.11. Unit HPO Personnel. 12

2.12. Unit Medical Director. 12

2.13. Unit PJs. 13

2.14. Unit Medical Materiel Manager (MMM). 14

2.15. Unit Independent Duty Medical Technician 16

Chapter 3—ADMINISTRATION OF MEDICAL ACTIVITIES **17**

3.1. Organization. 17

3.2. Credentialing & Privileging Documentation. 18

3.3. Quality Assurance (QA). 18

3.4. Operational Clinical Services (OCS). 18

3.5. Medical Information Management. 18
3.6. Infection Control Responsibilities. 19
3.7. Medication Dispensing Devices. 19
3.8. Ancillary Services. 19
3.9. Self-Assessment. 19

Chapter 4—TRAINING **20**

4.1. Training Requirements. 20
4.2. Administration of Medical Training. 20
4.3. AFSPECWAR Integrated Operational Support Personnel. 20
4.4. Unit Medical Directors. 20
4.5. Unit PJ Medical Training. 22

Chapter 5—AFSPECWAR MEDICAL EQUIPMENT AND SUPPLIES **23**

5.1. Budgeting & Funding for Medical Supplies. 23
5.2. Medical War Reserve Materiel (WRM). 23
5.3. Deviations and Waivers. 25
5.4. Funding for Supplies. 25

Chapter 6—PHARMACEUTICAL CONTROL AND STORAGE **26**

6.1. Specified Tasks. 26
6.2. Storage, Management, and Documentation. 26
6.3. Storage, Management, and Documentation of Controlled Medical Items in Support of AFSPECWAR Missions and Medical Coverage. 27
6.4. Issuance of Controlled Medical Items in Support of AFSPECWAR Missions and Medical Coverage. 28
6.5. Transportation of Controlled Medical Items. 28
6.6. Loss Reporting. 29
6.7. Controlled Medical Items Commercial Credit Return and Destruction. 29

Attachment 1—GLOSSARY OF REFERENCES AND SUPPORTING INFORMATION **30**

Attachment 2—HUMAN PERFORMANCE ADVISORY BOARD (HPAB) EXECUTIVE COUNCIL **36**

Attachment 3—PJ MEDICAL OPERATIONS ADVISORY BOARD (MOAB) EXECUTIVE COUNCIL **37**

Attachment 4—SAMPLE APPOINTMENT LETTER TEMPLATE **39**

Chapter 1

INTRODUCTION

1.1. Overview. AFSPECWAR Airmen comprise the Guardian Angel, Tactical Air Control Party, and the Special Tactics Weapon Systems. AFSPECWAR Medical Operations provides HPO for AFSPECWAR Airmen, integrated medical oversight of Pararescue (PJ) personnel, and enables medical support for global access, precision strike, and personnel recovery.

1.2. AFSPECWAR Medical Operations.

1.2.1. AFSPECWAR Medical Support. AFSPECWAR Medical Support consists of medical personnel, medical-support, and non-medical personnel who provide in-garrison medical care, HPO initiatives, and integrated physician oversight of AFSPECWAR medical operations known as Integrated Operational Support (IOS) IAW DAFMAN 10-211, *Integrated Operational Support (IOS)*. These functions are collectively referred to as AFSPECWAR IOS. **Note:** By definition, AFSPECWAR IOS are not Squadron Medical Elements who support aviation units; AFSPECWAR commanders and IOS personnel are not bound by Squadron Medical Element policies and guidance targeted towards aviation units.

1.2.1.1. In-garrison Medical Care. This integrated medical support team may consist of a physician, physician assistant, medic and/or IDMT who provides routine medical care in-garrison, such as routine medical care, annual and deployed health assessments, force health protection, and forward medical planning IAW Air Force Manual (AFMAN) 48-149, *Flight and Operational Medicine Program (FOMP)*, and DAFMAN 10-211.

1.2.1.2. HPO Team. Includes medical personnel and non-medical HPO assets to provide comprehensive readiness and preventive care, mental health support, early diagnosis and treatment of injuries, nutritional guidance, and overall health and performance optimization of the AFSPECWAR Airmen.

1.2.1.3. Integrated physician oversight of AFSPECWAR operational medicine. Physician oversight of medical operations for PJs includes but not limited to in-unit PJ medical skills sustainment training, medical training during assessment, selection, exercises or local skills sustainment, after action report (AAR)/mission report review for medical indications/treatment and medical equipment training/validation.

1.2.1.3.1. Oversees the coordination of medical logistics and equipment with the local medical treatment facility (MTF).

1.2.1.3.2. Oversight of PJ medical operations to include sustainment training, field training exercises, medical logistics and equipment, review medical interventions on after action reports and provide feedback.

1.2.2. Deployable Aeromedical Operational Support. The deployable Aeromedical Operational Support Team deploys with the AFSPECWAR deployable force element. The Aeromedical Operational Support Team is a three-person element; when deployed, it is comprised of a flight surgeon or physician assistant, independent duty medical technician (IDMT), and a physical therapist. When not deployed, the Aeromedical Operational Support Team is a component of the unit's AFSPECWAR IOS. The unit medical director provides medical oversight for all medical operations in garrison and for deployed medical operations.

Chapter 2

ROLES AND RESPONSIBILITIES

2.1. Air Force Surgeon General (AF/SG).

2.1.1. The AF/SG serves as the principal advisor on all health and medical matters of the USAF and United States Space Force under the authority of Title 10 United States Code Section 9036, *Surgeon General: Appointments; Duties*; Public Law (PL)115-232, *National Defense Authorization Act for Fiscal Year 2020*, Section 712, as amended by PL 116-92, *National Defense Authorization Act for Fiscal Year 2020*, Section 712(a); DoD Instruction (DoDI) 6025.13, *Medical Quality Assurance and Clinical Quality Management in the Military Health System*; and Headquarters Air Force Mission Directive (HAFMD) 1-48, *The Air Force Surgeon General.*

2.1.2. Oversees medical care provided by all DAF personnel. Ensures AFSPECWAR IOS and PJ medical operations will follow AF/SG guidance to ensure AFSPECWAR units have integrated physician oversight IAW DAFMAN 10-211.

2.1.3. Coordinates all DAF IOS programs to validate whether a program delivers clinical or clinical support services and must approve any program delivering clinical or clinical support services prior to any healthcare delivery.

2.1.4. Designates the AFSPECWAR Medical Director (AF/A34SG) as the lead agent for AFSPECWAR IOS.

2.2. Air Force Special Warfare Division (AF/A34S).

2.2.1. Executes AFSPECWAR relevant management responsibilities outlined in HAFMD 1-54, *Deputy Chief of Staff, Operations.*

2.2.2. Appoints the AF/A34SG as lead for AFSPECWAR medical policies and standardization related to PJ medical oversight, medical protocol, interoperability, HPO, and medical equipment.

2.2.3. Provides at a minimum, one O-6 level representative as AFSPECWAR medical subject matter expert to the United States Special Operations Command (USSOCOM) Board of Command Surgeons, the Manpower and Equipment Force Packaging (MEFPAK) conference, and the Air Force Medical Service Council meetings (or equivalent functional boards).

2.2.4. Serves as the final authority on all requests for waivers for initial and recurring PJ medical training, medical oversight, medical materiel, and AFSPECWAR IOS program implementation. Will advocate AFSPECWAR equities to Air Force Chief of Aerospace Medicine (AF/SG3P) on all AFSPECWAR SWA waivers or retention waivers related to medical standards, medical training, medical control authority, and HPO.

2.2.5. Briefs and coordinates with headquarters Air Force directorates and other officials or organizations on PJ medical operations, equipment, and training matters.

2.2.6. Adjudicates MAJCOM disparities pertaining to formal PJ medical training.

2.2.7. Coordinates with Air Education and Training Command, Operations and Communications (AETC/A3/6) and appropriate USSOCOM medical training organizations in the allocation of initial and recurring PJ medical training quotas.

2.2.8. Develops and prioritizes inspection requirements to ensure PJ medical training and equipment standards meet regulations. Refer to Department of the Air Force Instruction (DAFI) 90-302, *The Inspection System of the Department of the Air Force,* for inspection requirements.

2.2.9. Ensures processes and roles/responsibilities between headquarters Air Force organizations with PJ medical equities are defined and understood (e.g., safety management, equipment management).

2.3. Air Force Special Warfare Medical Director (AF/A34SG).

2.3.1. Develops policy and guidance for AFSPECWAR medical operations.

2.3.2. Communicates AFSPECWAR medical program requirements to the AF/SG and AF/A34S.

2.3.3. Integrates operations between the Air Force PJ Medical Director (PJ MD) and AFSPECWAR HPO Director on the application of standardized protocols, equipment cross-utilization and packages where applicable to increase cross-command medical interoperability.

2.3.4. Coordinates with AF/SG on AFSPECWAR medical standards and is a voting member of the quarterly AFMEDCOM medical standards working group, making recommendations on medical standards for AFSPECWAR Airmen.

2.3.5. Coordinates with AFMEDCOM and other military services for AFSPECWAR Airmen medical requirements to include waivers and exception to policy.

2.3.6. Directs requirements for AFSPECWAR medical officer training.

2.3.7. Directs requirements for AFSPECWAR Aeromedical Operational Support training.

2.3.8. Approves the AFSPECWAR medical officer special experience identifier. May be delegated in writing to the PJ MD.

2.3.9. Coordinates AFSPECWAR medical interoperability and standardization to include point of care medical stabilization, prolonged field care, and patient transport along the evacuation continuum.

2.3.10. Ensures coordination and integration with relevant stakeholders of the PJ Medical Operations Advisory Board (MOAB), and the Human Performance Advisory Board (HPAB).

2.3.11. Appoints in writing the Air Force Pararescue Medical Materiel Manager (PJ MMM) as an additional duty, with written approval of that member’s commander.

2.3.12. Directs semi-annual review and validation of the HPAB Executive Council for approval by the AF/A34S.

2.3.13. Directs PJ MD’s semi-annual review and validation of the PJ MOAB Executive Council for approval by the AF/A34S.

2.4. Air Force Special Warfare Human Performance Optimization Director (HPO Director).

2.4.1. Develops policy and executes AFSPECWAR HPO doctrine, policy, and guidance to the AFSPECWAR community.

2.4.2. Provides oversight and guidance to both medical and non-medical HPO personnel to support full spectrum operations.

2.4.3. Maintains AFSPECWAR total force HPO personnel roster.

2.4.4. Coordinates HPO and related programs with other military services.

2.4.5. Communicates requirements to ensure interoperability and standardization of programs, equipment, and personnel for AF/A34S and the 1Z/19Z Career Field Managers.

2.4.6. Chairs, maintains, and updates the HPAB charter. See **Attachment 2**.

2.4.7. Represents AFSPECWAR IOS HPO programs as a voting member in the AF/SG IOS working groups and actively participates in Department of War (DoW) and civilian professional organizations.

2.4.8. Provides oversight and coordinates HPO research and development with subject matter experts and other HPO data analysis personnel IAW the HPAB charter and policy guidance.

2.4.9. Reviews all HPO research, data, and metrics from all units.

2.4.10. Develops curriculum for AFSPECWAR Medical Officer Course and conducts or delegates instructors for the HPO curriculum biannually.

2.4.11. Coordinates and communicates HPO requirements with Air Force Chief of Chaplains (AF/HC) no less than annually. This coordination should be formalized through a Memorandum of Agreement (MOA) to clearly define roles, responsibilities, and support provided for spiritual and mental health resiliency.

2.4.12. Provides functional site visits when requested by commanders to review their HPO program.

2.4.13. Maintains Air Force specialty code (AFSC) credentials and currency IAW the Department of the Air Force Officer Classification Directory (DAFOCD).

2.4.14. Adheres to DAFMAN 10-211 policy and executes requirements to the AFSPECWAR community.

2.4.15. Identifies, develops, and executes onboarding, initial, and ongoing E&T requirements for HPO personnel.

2.4.16. Must comply with implementation of Clinical Quality Management (CQM) programs outlined in DoDI 6025.13 and AF/SG guidance at all IOS locations.

2.5. Air Force Pararescue Medical Director (PJ MD).

2.5.1. Develops and executes policy and guidance for Pararescue Medical Scope of Practice, and PJ medical support to the AFSPECWAR deployable force element.

2.5.2. Ensures the *USAF Pararescue Medical Operations Handbook (PJ Med Handbook)* and other written specified protocols are current and accurately reflect appropriate standards of care for PJ medical mission sets.

2.5.3. Coordinates memorandums of agreement (MOAs) and training affiliation agreements as needed for AFSPECWAR PJ medical currency.

2.5.4. Oversees PJ patient registry and reviews all AARs and consolidated mission reports as part of a total force quality improvement and performance improvement program.

2.5.5. Provides medical oversight and guidance for all phases of the PJ medical training including initial, refresher, and sustainment training.

2.5.6. Represents PJ and AFSPECWAR IOS equities to other military services, combatant commands, and civilian equivalent surgeons.

2.5.7. Represents PJ medical equities in DoW and civilian professional medical organizations pertinent to AFSPECWAR medicine to include, but not exclusive to the Joint Trauma System (JTS), Committee on Tactical Combat Casualty Care (TCCC), USSOCOM Board of Command Surgeons, and National Registry of Emergency Medical Technicians (NREMT).

2.5.8. Maintains and updates the PJ MOAB charter. The PJ MOAB Executive Council will review and validate the charter biennially.

2.5.9. Chairs the PJ MOAB Executive Council. See **Attachment 3**.

2.5.10. Using the PJ MOAB function, coordinates with MAJCOM Command Surgeons on standardized medical protocols, equipment cross-utilization, and medical kits as required and applicable to enhance PJ and Special Operations Forces interoperability.

2.5.11. Coordinates curriculum for the AFSPECWAR Medical Officer Course with the HPO Director and AF/A34SG.

2.5.12. Coordinates PJ medical scope of practice and protocol updates with the AF/SG Emergency Medical Services (EMS) Consultant and AFMEDCOM Chief of Quality.

2.5.13. Consults with the PJ Career Field Manager and the PJ MMM for approval of safe-to-fly medical equipment.

2.5.14. Must comply with implementation of CQM programs outlined in DoDI 6025.13 and AF/SG guidance at all IOS locations.

2.6. Air Force Pararescue Medical Programs Manager (PJ MPM).

2.6.1. Coordinates policy integration of medical materiel and logistics to ensure operational interoperability and tactical applicability for the PJ career field.

2.6.2. Coordinates medical scope of practice standardization as directed by the PJ MD for operational applicability and interoperability.

2.6.3. Coordinates medical oversight and guidance for all phases of PJ medical training including initial, refresher, and sustainment requirements.

2.6.4. Maintains the PJ patient registry and reviews all AARs and consolidated mission reports as part of a total force quality improvement and performance improvement program.

2.6.5. Reviews monthly medical documentation from MAJCOM's units, ensuring standardized medical documentation and logging requirements.

2.6.6. Plans and executes the PJ MOAB IAW the charter.

2.6.7. Updates and distributes the *PJ Med Handbook* and equivalent guidance every two years, to ensure the handbook represents the most up to date information to be reviewed and presented at least annually at the PJ MOAB.

2.6.8. Represents PJ medical equities with DoW and civilian professional medical organizations pertinent to AFSPECWAR medicine and tactics integration to include, but not exclusive to, the JTS and its subsidiary committees, Joint Enlisted Medical Advisory Council, Special Operations Medical Association, and NREMT.

2.6.9. Serves as Pararescue liaison to the USSOCOM Surgeon's Office and the Air Force State Office of Emergency Medical Services (AFSOEMS).

2.6.10. Serves as Pararescue liaison to the DAF Animal Research Oversight and Compliance Office.

2.7. Air Force Pararescue Medical Materiel Manager (PJ MMM).

2.7.1. Provides policy and guidance in coordination with the PJ MPM for allowance standards (AS) and assemblages for the PJ career field and presents updates at the PJ MOAB.

2.7.2. Oversees selection, standardization, evaluation, testing, and approval for medical equipment. **Note:** Medical equipment items (electronic and medical grade oxygen) must be flight certified as safe-to-fly. A safe-to-fly list of equipment is maintained by AFMEDCOM Medical Logistics Training (AFMEDCOM/SG4T) and can be found at **https://medlog.us.af.mil/apps/equipcat/**. Users may access the site using a Common Access Card. Equipment requiring test and evaluation will be staffed through the appropriate MAJCOM directorates.

2.7.3. Coordinates immediate need/emergent items for flight approval/waiver.

2.7.4. Conducts AS review triennially (once every three years). Review will be sent to the AF/A34SG and presented at the PJ MOAB prior to sending validated changes to AFMEDCOM for final approval.

2.7.5. Documents and catalogues item selection rationale and evidence in the periodic reviews to inform current and future modernization efforts and AS upgrades on the Guardian Angel SharePoint Page (**https://usaf.dps.mil/teams/13306/medical/SitePages/Home.aspx**).

2.7.6. Coordinates deployment requirements with MEFPAK office and Air Force Medical Logistics Operations Center to establish Department of Defense Activity Address Code for new contingency locations.

2.7.7. Addresses re-supply issues within the area of responsibility, Theater Lead Agent for medical material and Air Force Medical Logistics Operations Center. For expedited delivery and urgent requirements, address re-supply with the nearest or supporting hub location.

2.8. MAJCOM Headquarters Staff.

2.8.1. Coordinates MOAs as needed to ensure assigned AFSPECWAR IOS personnel meet requirements IAW DoDI 6025.13.

2.8.2. Coordinates memorandums with MAJCOM Surgeon General staff, wing project officers and AF/A34SG, PJ MD, and HPO Director.

2.8.3. Serves as advisors and representatives to AF/A34S to subordinate commands.

2.8.4. Communicates requirements for AFSPECWAR IOS mission continuity and manpower stabilization to AF/A34SG.

2.8.5. Coordinates and supports AF/A34S policy and guidance for all AFSPECWAR IOS personnel.

2.8.6. Provides AFSPECWAR IOS expertise to all 1Z/19Z MAJCOM Functional Managers.

2.8.7. Coordinates and communicates resource requirements across functional area managers and AF/A34S for sustainment of current and AFSPECWAR IOS mission capabilities.

2.8.8. Conducts strategic assessments and analysis of integrated programs and facilities, validates manpower and programming requirements, and resources PJ mission requirements through the Program Objective Memorandum process.

2.8.9. Coordinates research and development with subject matter experts, monitors human performance tracking software programs, and validates measures of performance and effectiveness data and ensures coordination with the HPO Director for submission to AF/A34S.

2.8.10. Maintains AFSC credentials and currency IAW the DAFOCD and the Department of the Air Force Enlisted Classification Directory (DAFECD).

2.8.11. Provides voting members for the HPAB and PJ MOAB Executive Councils. See **Attachment 2** and **Attachment 3**.

2.9. Major Command Surgeon (MAJCOM/SG).

2.9.1. Communicates specific operational and functional medical requirements to AF/A34SG and appropriate MAJCOM Operations Division (MAJCOM/A3) staff.

2.9.2. Implements and oversees MAJCOM unique policies for PJ medical training, PJ medical oversight, and medical care documentation requirements in coordination with the PJ MAJCOM functional manager (PJ MFM).

2.9.3. Provides medical planning assistance to include coordination of Annex Q (Medical) and Annex C (Operations) for all relevant operational plans and operational orders.

2.9.4. Coordinates training requirements for AFSPECWAR IOS personnel assigned to Aeromedical Operational Support for AFSPECWAR deployable force elements with AF/A34SG.

2.9.5. Liaises with the medical treatment facility commander (MDG/CC) on IOS programs for AFSPECWAR units.

2.9.6. Coordinates with AF/A34SG on background, education, and experience requirements for AFSPECWAR IOS medical and medical support personnel.

2.9.7. Monitors and assesses PJ medical and AFSPECWAR IOS Management Internal Control Toolset (MICT) data from units to maintain situational awareness of potential areas of non-compliance. Guidance on MICT is provided in DAFI 90-302.

2.9.8. Appoints a MAJCOM PJ MMM, preferably with prior experience in a Guardian Angel unit, to coordinate with the AFSPECWAR PJ MMM on War Reserve Materiel (WRM) and AS procurement, maintenance, and utilization. Decisions impacting PJ mission execution (e.g., equipment testing, flight certification, or deployment resupply) must be made in coordination with the PJ MFM. **Note:** For the Air Force Reserve, PJ MMM will be appointed in coordination with Air Force Reserve Command, Directorate of Operations, Plans, and Requirements (AFRC/A3J).

2.9.9. Provides necessary assistance to PJ MFMs during inspections and staff assistance visits.

2.10. Unit Squadron Commanders (SQ/CC).

2.10.1. Appoint the unit medical director in writing and email the appointment to the PJ MD and local Chief of Medical Staff within 30 days of arrival on station. Maintain a copy in each PJ's medical training record.

2.10.2. Appoint the unit Medical Material Manager (MMM) in writing and email the appointment to the PJ MPM and local MTF within 30 days of arrival on station.

2.10.3. Coordinate monthly with AFSPECWAR IOS team to ensure mission impacting operational and medical concerns are addressed.

2.10.4. Seek support from the host MTF to acquire and maintain all medical materiel required for mission support.

2.10.5. Seek support from the host MTF for credentialing and privileging of AFSPECWAR IOS providers.

2.10.6. Ensure unit medical personnel have sufficient time allotted as well as adequate clinical opportunities to meet clinical currency requirements.

2.10.7. Appoint in writing a medical logistician as the responsible WRM Project Officer.

2.10.8. Develop operating instructions for the maintenance and deployment of WRM and unit type code mobility assets.

2.10.9. Provide appropriate medical materiel storage areas.

2.10.9.1. Coordinate and establish MOA with the local medical facility, as appropriate, concerning storage and maintenance of WRM and medical maintenance.

2.10.9.2. Establish limited access medical materiel storage areas for non-WRM materiel and maintain a signed personnel authorized access roster.

2.10.9.3. Ensure appropriate security and environmental controls for WRM and medical equipment storage. **Note:** Security must be provided for alert kits and equipment.

2.10.9.4. Ensure all controlled substances meet security and environmental control requirements IAW Title 21, Code of Federal Regulations, Part 1301.71 - 1301.76, *Security Requirements,* current edition.

2.10.10. Coordinate and supports unit's medical logistician access to Defense Medical Logistics Support System (DMLSS).

2.10.11. Ensure medical after-action reports and consolidated mission reports involving patient care regardless of mission set or unit being supported. Mission reports should be given to the PJ patient registry and JTS registry as part of a Total Force quality improvement and performance improvement program within five business days of mission completion (**https://jts.health.mil/index.cfm/documents/forms_after_action**).

2.10.12. Ensure unit-assigned AFSPECWAR deployable force element personnel attend required training IAW **Chapter 4** within one year of arrival on station and prior to any scheduled deployments.

2.10.13. Coordinate with host MTF to ensure AFSPECWAR IOS are meeting appropriate Clinical Quality Management requirements. For those locations not co-located with an MTF, follow DoDI 6025.13, DHA-PM 6025.13, *Clinical Quality Management in the Military Health System,* and AFMEDCOM Clinical Quality Management guidance.

2.11. Unit HPO Personnel.

2.11.1. Executes AFSPECWAR IOS within the assigned unit according to DAFMAN 10-211 and AF/A34S policy and guidance.

2.11.2. Delivers monthly data reports to the HPO Director via approved data management system.

2.11.3. Provides semi-annual staffing report to AFSPECWAR HPO Director.

2.11.4. Reports AFSPECWAR IOS data sets and research or analysis that may result in publication to the HPO Director via email within 30 days of collection.

2.11.5. Complies with training requirements, as outlined in **Chapter 4**.

2.11.6. Maintains currency and Clinical Quality Management requirements IAW governing body policies (DHA, DAF, AF/HC, all applicable national certification, and licensing boards).

2.12. Unit Medical Director.

2.12.1. Coordinates with the local Medical Group (MDG) Chief of Aerospace Medicine (MDG/SGP) on guidance and procedures as it relates to unit medical providers and staff.

2.12.2. Ensures all unit medical personnel maintain professional staff obligations with host MTF as required.

2.12.3. Provides medical oversight for duties covered by a unit-aligned or local physician medical director IAW DoW, Department of Transportation, and USAF directives to oversee the medical training, qualification, and practice of unit-assigned paramedics to include PJs.

2.12.3.1. Serves as clinical supervisor for PJ personnel to include oversight of Paramedic and other applicable certifications, medical training management, and medical currency/proficiency of attached PJs. The unit medical director will provide clinical supervision, oversight, and guidance to the AFSPECWAR unit Noncommissioned Officer of Medical Training.

2.12.3.2. Ensures PJ medical currency by working with the unit standardization and evaluation team by reviewing and auditing PJ medical training status.

2.12.4. Implements and track required PJ medical recertifications to include maintenance of the NREMT certification biennially (every two years).

2.12.5. Utilizes the *PJ Med Handbook* as appropriate for the medical/tactical situation and provide appropriate supervision while working directly with PJs as a medical preceptor to diagnose and treat illness or injuries. This is done in a training capacity and only to develop and maintain clinical proficiency required to perform small unit care in a deployed environment IAW **Chapter 4**.

2.12.6. Collects, reviews, and organizes PJ medical treatment records and patient documentation for ensuring appropriate patient documentation.

2.12.7. Documents PJ medical infractions and recommend corrective action through the member's chain of command.

2.12.8. Ensures documentation for all prehospital patient encounters, submission of patient encounter documentation to JTS and AF/A34S utilizing the DoD Form (DD)1380, *Tactical Combat Casualty Care (TCCC) Card*, and Department of the Army Form (DA) 4700, *Medical Record-Supplemental Medical Data*, or other approved/electronic means of documentation (**https://jts.health.mil/index.cfm/documents/forms_after_action**).

2.12.9. Completes required training outlined in **Chapter 4**.

2.12.10. Ensures assigned PJs are trained on proper storage of controlled substances, medical equipment, and have access to appropriate supplies.

2.12.11. Completes a controlled medication letter, when required.

2.12.12. Appoints in writing the Medical Training Coordinator and Noncommissioned Officer of Medical Training. Guidance on Medical Training Coordinator duties is provided in AFMAN 10-3500, Volume 1, *AFSPECWAR Training Program*.

2.12.13. Maintains AFSC credentials, primary board certification, and currency IAW the DAFOCD.

2.12.14. Notifies the AFSOEMS, in writing, of medical incident investigation cases involving the performance of prehospital duties as a certified EMS clinician.

2.12.15. Notifies the AFSOEMS, in writing, of any administrative investigations or adverse actions that result in removal of a PJ or IDMT from patient care, to include violations of the NREMT's published Code of Conduct.

2.12.16. Integrates unit medical requirements with standard medical operational plans and provide input for exercise and contingency planning, as required, to determine supply and equipment requirements.

2.12.17. Conducts in-garrison medical care for unit assigned personnel but will defer formal empanelment of unit members and dependents to the local MTF.

2.13. Unit PJs.

2.13.1. Ensure standardized packing and maintenance of medical kits IAW standard operating procedures at unit or mission level.

2.13.2. Ensure proper storage and accountability of issued controlled substances and medication kits.

2.13.3. Ensure medical after-action reports and consolidated mission reports are submitted to the JTS, PJ MD, and PJ MPM within five days of mission completion. **Note:** Utilize JTS document templates and submit IAW the guidance found on the JTS AAR website at **https://jts.health.mil/index.cfm/documents/forms_after_action**.

2.13.4. Work with squadron medical support personnel for local area training medical coverage and planning. **Note:** PJs are authorized to provide care in an emergency or medically urgent situation such as during range coverage. Refer to AFMAN 10-3511, *Pararescue and Combat Rescue Officer Operations*, and AFI 10-3503, *Personnel Parachute Program,* for guidance.

2.13.5. During dynamic or high-risk operations, all personnel will apply Real-Time Risk Management. Every team member, regardless of rank or position, has the authority and responsibility to initiate a 'Knock-it-off' or 'Timeout' call to immediately halt any action if they perceive an unmitigated or unforeseen hazardous situation.

2.13.6. Coordinates with physician/medical director to ensure proper medical documentation occurs when providing medical care and it is reported IAW with policy in the DoW approved Electronic Health Record (EHR) system.

2.14. Unit Medical Materiel Manager (MMM). Appointed in writing by the squadron commander as the Medical Property Custodian. The appointee(s) should be a Medical Logistician (4A1X1). In the event a medical logistician is not assigned to the unit, IDMT (4N0X1C) or an Aerospace Medical Service Specialist (4N0X1) is able to execute medical materiel duties and responsibilities until a 4A1X1 is available. Appointee(s) will provide a copy of the letter to the host medical logistics for filing and updating the custodian point of contact information in DMLSS. Unit MMM will:

2.14.1. Assist squadron commanders in ensuring proper storage facilities are provided, evaluate storage area adequacy to prevent fire theft, pilferage, and ensure proper environmental controls (i.e., clean and organized).

2.14.2. Oversee alert kit, equipment, and medications storage and control by securing all medical kits in a locked cabinet, cage or secured room when not on alert or signed out and in the possession of a PJ, medical director, or IDMT.

2.14.3. Manage and operate the medical logistics section as the focal point for all medical materiel requirements.

2.14.4. Take custody and assume responsibility of all property receipted and signed on the host MTF medical materiel stock record account or otherwise entrusted to the care of the unit medical logistics section.

2.14.5. Order, receive, store, safeguard, issue, inventory, turn-in, and dispose of medical materiel, and maintain control of records of the same. (e.g., receipts, issues, turn-ins, destructions, inspections, inventories, and out-shipments). See **Chapter 6**.

2.14.6. Inspect and control all medical materiel and equipment required. See **Chapter 6**.

2.14.7. Maintain on-hand medical supplies and equipment in serviceable condition and at levels that reduce capital investment inventory and minimize generating excess.

2.14.8. Check materiel expiration dates monthly to ensure maximum rotation of expiring items to minimize waste.

2.14.9. Secure and dispose of all single use medical items such as needles and syringes.

2.14.9.1. Ensure proper disposal of all expired materials are unrecognizable as well as unusable.

2.14.9.2. Establish and monitor proper storage and transportation of used syringe needles, scalpel, blades, or any products that can cut or puncture personnel are in the appropriate sharps containers during field use by personnel in the unit.

2.14.10. Coordinate destruction methods with the Base Civil Engineering Environmental Manager and provide a signed and dated record of receipt, documenting the transfer of materiel.

2.14.11. Ensure all unit-owned medical equipment is in operational condition. Prohibit the use of non-operational equipment to include items used during training.

2.14.12. Review the Equipment Replacement Report quarterly to determine equipment replacement requirements.

2.14.13. Ensure deployment readiness of unit owned medical equipment and coordinate maintenance requirements of applicable equipment with the host MTF biomedical equipment technician or contracted entities IAW applicable agreement with the MTF. Implement and assess compliance with preparation guidance in AFI 10-403, *Deployment Planning and Execution*, and AFMAN 24-604, *Preparing Hazardous Materials for Military Air Shipments,* for WRM mobilization and transportation.

2.14.14. Coordinate with the unit medical director and resource advisor to develop and maintain the medical funding program to include monitoring, commitment, and obligation of medical logistics funds and status of the medical logistics operating program.

2.14.15. Comply with applicable host MTF quality assurance (QA) procedures and maintain a local QA file containing all applicable QA messages, device recalls, other QA notices, and affected items turn-in documents.

2.14.16. Coordinate with the PJ MMM for activation of medical WRM assets, if applicable to the deployed location. Upon approval of activation request, the unit MMM will coordinate and assist the host WRM Project Manager in the inventory and readying of the asset(s).

2.14.17. Coordinate medical logistics MOAs or memorandums of understanding (MOUs) with the host MTF Resource Management office or any support agreements with non-MTF entities (Army/Navy Depots, Theater Lead Agent for medical material, etc.). AFI 25-201, *Intra-Service, Intra-Agency, and Inter-Agency Support Agreements Procedures,* provides guidance in completing MOAs and MOUs.

2.15. Unit Independent Duty Medical Technician (IDMT).

2.15.1. Maintains current Medical Training Coordinator/Noncommissioned Officer of Medical Training and Squadron Medical Director appointment letter(s) and submit a copy to the AF EMS Program Manager. **Note:** IDMT requirements are not applicable to Air Reserve Component (ARC).

2.15.2. Completes TCCC Tier 4 Training and become a certified instructor within six months of assignment (12-months for ARC).

2.15.3. Completes training and certify as an instructor in Pediatric Advanced Life Support, Advanced Cardiac Life Support, and Basic Life Support within six months of assignment.

2.15.4. Completes training and is certified as an instructor in National Association of Emergency Medical Services Educators Level 1 within six months of assignment.

2.15.5. Organizes, conducts, and tracks EMS continued education IAW current NREMT, AFSOEMS, AFMAN10-3500V1, and local requirements. **Notes:** The unit IDMT will complete all training required to achieve the "Training Officer" role with NREMT. The unit IDMT ensures all unit-level EMS continued education is directly overseen by a certified AF EMS instructor. The unit IDMT performs skills training and validation for all assigned NREMT personnel and reports all validation metrics to the Squadron Medical Director.

2.15.6. Ensures all unit NREMT certified personnel are affiliated with the Special Operations Center for Medical Integration and Development (SOCMID) in the NREMT database and coordinates attendance and training completion at SOCMID, as required.

2.15.7. Tracks and reports medical training go/no-go criteria to the unit commander and unit Operations Officer.

2.15.8. Maintains educational material/textbooks.

2.15.9. Attends local, state, and/or national medical symposiums/conferences as a means of professional development.

2.15.10. Coordinates and facilitates reentry for members with an expired NREMT certification and/or certification extensions with the AFSOEMS. Coordinates with the PJ MPM prior to initiating paramedic reentry requirements for a lapsed certification.

2.15.11. Completes all medical risk assessment training and provides medical risk assessments in support of unit activities and recommends/implements courses of action to mitigate risk. Refer to AFI 44-103, *The Air Force Independent Duty Medical Technician Program,* Chapters 7 and 8 for additional guidance.

2.15.12. The IDMT will conduct medical risk assessments for unit activities utilizing the doctrinal 5-Step RM Process: (1) Identify the Hazards (2) Assess the Hazards (3) Develop Controls & Make Decisions (4) Implement Controls (5) Supervise & Evaluate.

Chapter 3

ADMINISTRATION OF MEDICAL ACTIVITIES

3.1. Organization.

3.1.1. AFSPECWAR IOS personnel deliver healthcare, both in-garrison and while deployed. When in-garrison, operational medical personnel will prioritize maintenance of clinical proficiency, meeting medical readiness training, administrative requirements, and maintenance of clinical credentials and privileges. Meeting and sustaining these requirements contained herein qualifies AFSPECWAR's operational medical personnel for deployment. Medical personnel will not be assigned duties that detract from their primary role as a medical provider. **(T-1)**

3.1.2. AFSPECWAR's operational medical personnel must be prepared for operational deployment. Field medical care and pre-hospital trauma care is the primary focus for AFSPECWAR medical personnel. Clinical competency and proficiency are the most important and critical qualifications operational medics bring to the fight, and these skills are extraordinarily perishable. All AFSPECWAR operational medical personnel, without exception, must meet or exceed their duty clinical requirements within their scope of practice. Refer to DAFMAN 10-211 and Comprehensive Medical Readiness Program checklists (Cat 1 & Cat 2) for additional guidance. In order to maintain these perishable qualifications, the primary emphasis of in-garrison activities will be prioritized to the delivery of safe and effective healthcare in clinical and hospital settings.

3.1.3. All AFSPECWAR IOS personnel will comply with applicable DAF, DoW, and DHA requirements that govern the delivery of healthcare IAW DoDI 6025.13, DHA-PM 6025.13, AFI 44-119, *Medical Quality Operations,* and DAFMAN 10-211. **(T-0)**

3.1.3.1. All medical personnel must meet and maintain credentialing and privileging requirements with the host MTF.

3.1.3.2. Unit commanders will ensure assigned IOS personnel meet clinical currency and Comprehensive Medical Readiness Program requirements. **(T-1)**

3.1.3.3. AFSPECWAR medical personnel who fail to meet clinical competency and currency requirements may be subject to clinical adverse action IAW DoD Instruction (DoDI) 6025.13, and AFI 44-119.

3.1.3.3.1. Employment may be denied or terminated, if IOS personnel do not meet credential and privileging requirements. **(T-0)**

3.1.3.3.2. The hiring manager (or supervisor) will work with installation civilian personnel office or the contracting officer's representative to determine appropriate action.

3.1.3.4. All medical personnel will complete all training requirements for Credentialing & Privileging, to include MTF professional staff meetings.

3.1.4. For in-garrison chain of command, the unit SQ/CC has administrative control, operational control, and tactical control of their line funded personnel and programs, and punitive authority IAW the Uniform Code of Military Justice over operational medical personnel. The rating chain will be through the operational squadron.

3.2. Credentialing & Privileging Documentation. All medical personnel will maintain a current copy of their medical credentials in the credentialing system of record and in their deployment folder. **(T-2)**

3.2.1. IOS medical personnel must meet and maintain all their credentialing and privileging requirements IAW their employment and readiness requirements. **(T-0)** Guidance on credentialing and privileging requirements are provided in DoDI 6025.13 and DHA-PM 6025.13, Volume 4 and Volume 8.

3.2.2. The senior AFSPECWAR physician at the deployed location will review Inter-facility Credentials Transfer Briefs, or Enterprise Privilege DHA 456, and IDMT certification documents. **(T-1)**

3.2.3. Deployable AFSPECWAR medical personnel must have and will maintain a deployment folder with copies of current medical credentials.

3.2.3.1. IDMTs will maintain a copy of their current, initial, and sustainment training documentation via the electronic Total Force Training Record or an approved electronic equivalent. **(T-2)** A hard copy of these documents will be maintained in their deployment folder.

3.2.3.2. Deployed medical personnel will provide co-located supporting expeditionary medical facilities a copy of their deployed medical folder.

3.2.4. IOS providers will meet credentialing and privileging requirements prior to delivery of clinical care in IOS locations.

3.3. Quality Assurance (QA). The unit's medical director is responsible for medical quality assurance.

3.3.1. QA will include peer review as described by the credentialing facility, both in garrison and in deployed settings.

3.3.2. IOS programs and personnel will operate in a manner that ensures compliance with all appropriate procedures and guidance. Reference DoDI 6025.13, AFI 44-119, and DHA-PM 6025.13, Volumes 1-8 for further guidance.

3.4. Operational Clinical Services (OCS). IOS provides medical and physical therapy, nutrition, and mental healthcare in locations outside the MTFs in-garrison and deployed locations called OCS locations. IOS will adhere to clinical quality management guidelines IAW building codes, DoDI 6025.13, DAFMAN 10-211, and other DoW related policies.

3.5. Medical Information Management. All in-garrison patient encounters performed by IOS medical personnel will be documented and closed in an EHR within 72-hours of rendered care IAW DHA-PM 6025.02V1. For encounters in a deployed or disconnected environment, documentation will be closed within 72-hours of returning to a location with EHR systems access. **(T-0)** Failure to appropriately document patient encounters both in-garrison and deployed can seriously jeopardize patient safety and quality of care.

3.6. Infection Control Responsibilities.

3.6.1. The unit medical director will ensure an active and current infection prevention and control program at all OCS locations. **(T-2)**

3.6.2. Guidance on infection prevention and control program is provided in AFI 44-108, *Infection Prevention and Control Program*, and DAFMAN 48-105, *Public Health Surveillance*.

3.7. Medication Dispensing Devices. IOS programs operating outside of the MTFs will not use medical dispensing devices for storage or dispensing of controlled substances. Controlled substances will only be dispensed via the MTFs outlined in **Chapter 6**.

3.8. Ancillary Services. The local MTF may provide support to the IOS programs in obtaining lab and radiology ancillary tests IAW DHA and DAF policies.

3.9. Self-Assessment. Self-assessment will be completed in MICT. Guidance on MICT is provided in DAFI 90-302.

3.9.1. At a minimum, two self-assessments are required annually. Line medical leaders will perform a self-assessment within 60 days of assuming the leadership role. (**T-2)**

3.9.2. The self-assessment program is one element of the quality program but does not cover all guidance applicable to the provision of medical care.

3.9.3. All guidance applicable to the provision of medical care by Air Force medical personnel also applies to the care provided by IOS medical and non-medical personnel. MICT inspections are different than operational quality inspections for IOS programs and personnel.

Chapter 4

TRAINING

4.1. Training Requirements. AFSPECWAR operational medical personnel require training that exceeds traditional career field requirements to support AFSPECWAR deployable force elements in austere, remote, or contested environments.

4.2. Administration of Medical Training.

4.2.1. AFSPECWAR IOS medical personnel will meet DAF Medical Readiness Program requirements for uniformed medical personnel described in DAFI 41-106, *Medical Readiness Program*. Uniformed medical personnel will maintain their currency and mission ready status with their respective Comprehensive Medical Readiness Program (Cat 1 & Cat 2). **(T-0)**

4.2.2. MAJCOM Chief of Medical Readiness (MAJCOM/SGX) is the OPR for Medical Readiness Reporting. Medical personnel training must be documented in Medical Readiness Decision Support System (MRDSS) located at **https://mrdss.cce.af.mil/**. Guidance on MRDSS is provided in DAFI 41-106. The unit Medical Training Coordinator will coordinate training with the MAJCOM/SGX and monitor MRDSS and Defense Readiness Reporting System (on SECRET Internet Protocol Router Network) monthly for accuracy. **(T-2)**

4.3. AFSPECWAR Integrated Operational Support Personnel.

4.3.1. All uniformed IOS HPO medical and non-medical personnel will complete the Medical Planners Course within 12-months of assignment to a unit supporting an AFSPECWAR deployable force element.

4.3.2. All uniformed IOS HPO medical and non-medical personnel will complete the AFSPECWAR Medical Officer Course within 12-months of assignment to the unit. **(T-1)**

4.3.3. All Non-medical HPO personnel must maintain licensure/certification specific to their profession. **(T-1)**

4.4. Unit Medical Directors.

4.4.1. Unit medical directors must provide full-spectrum aeromedical and operational support for AFSPECWAR personnel. **(T-1)** In-garrison, this includes routine medical care, periodic health assessments, waivers, HPO support, occupational visits, exercise support, medical surveillance, and intelligence review as needed to support the commander and mission. Of note, PJs are highly trained in forward austere medical care. Their skill and training in point of injury care may exceed that of the unit medical director. **Note:** ARC unit medical directors will not provide routine in-garrison medical care or oversee medical training unless the medical director is in an appropriate official duty status and working under the authority of the MTF with activation of clinical privileges.

4.4.2. Unit medical directors will be a residency trained, board certified or board eligible, and credentialed and privileged physician appointed in writing by the unit SQ/CC, forwarded to the PJ MD and the Medical Group Chief of Staff (MDG/SGH). **(T-1)**

4.4.3. The unit medical director will be an Aerospace Medical Primary Course graduate and hold a 48A/48R, Flight Surgeon, AFSC (this includes all PJ medical training platforms). **(T-1)** Additionally, a copy will be filed in each PJ's medical training record. See **Attachment 4** for a sample template.

4.4.4. To support PJ training and skill sustainment, unit medical directors may require additional training in the following subjects:

4.4.4.1. Initial Trauma Care.

4.4.4.2. Prolonged Casualty Care.

4.4.4.3. Casualty Evacuation.

4.4.4.4. Mass Casualty Incident Response.

4.4.4.5. Special Operations Forces Medical Skills Development.

4.4.4.6. Access and interpret medical intelligence and surveillance reports to provide line commanders guidance on chemical, biological, radiological, and nuclear response.

4.4.5. Attending formal courses, maintaining clinical currency, participating in integrated training exercises, and off duty employment where authorized, may also be a method of obtaining the foundational experience needed to become a unit medical director.

4.4.6. Deployable Minimum Training Requirements. Failure to meet training requirements will result in a non-mission ready recommendation to the unit commander.

4.4.6.1. Complete the AFSPECWAR Medical Officer Course within 12 months of assignment (24-months for ARC). **(T-1)**

4.4.6.2. Complete TCCC Tier 4, Advanced Tactical Medical Provider course within 12 months of assignment (24-months for ARC). **(T-1)**

4.4.6.3. Attend Center for Sustainment of Trauma and Readiness Skills every 24 months if unable to meet the American College of Surgeons recommendation to manage 35 multi-trauma resuscitations per year. **(T-1) Exception:** Not required for ARC medical directors who are actively working as credentialed emergency medicine specialists or anesthesiologists that meet the AFSC requirements through civilian practice.

4.4.6.4. Complete casualty evacuation training every 12 months (24-months for ARC). **(T-1)** This may be accomplished with a formal course, real world casualty evacuation missions, scenario based training, or simulated flights.

4.4.6.5. Attend Level-C Search, Survival, Evasion, Resistance and Escape (SERE) training, Rotary Wing Egress, and any other aircrew-specific training to ensure mission readiness and deployability. **(T-1)**

4.4.7. Unit medical director clinical competency and proficiency.

4.4.7.1. Clinical competency and proficiency are perishable and AFSPECWAR medical personnel must meet or exceed their duty requirements by consistently maintaining their competency and proficiency. **(T-2)**

4.4.7.1.1. A minimum of 20% of the medical director's available time per month (approximately one day per week) must be in the performance of clinical duties whether in the MTF or IOS unit operations. Clinical duties include, but are not limited to flight medicine, base operational medical cell, family medicine, urgent care, emergency medicine, pediatrics, radiology, and specialty clinics. This is not applicable for ARC medical directors who perform clinical duties in their civilian occupation.

4.4.7.1.2. Clinic schedules will be developed with the approval of the medical director's chain of command and in coordination with the host MTF or clinic at least 30 calendar days prior to clinical duties in the host MTF or clinic.

4.4.7.2. All medical directors or designees shall write narrative summaries, waivers and aeromedical summaries for their assigned personnel.

4.5. Unit PJ Medical Training.

4.5.1. PJs may not independently provide medical care in-garrison. However, PJs are encouraged to use the *PJ Med Handbook* protocols under the direct supervision of their unit medical director in a sick call setting in an IOS unit. The intent of supervised in-garrison care for PJs are for clinical training rather than to meet MTF appointment availability requirements. All medical care in-garrison will be documented in the EHR.

4.5.1.1. While functioning in this training capacity, the PJ will complete a medical note which will be reviewed and signed by their physician medical director or medical director-appointed physician preceptor. **(T-1)**

4.5.1.2. Medical skills and *PJ Med Handbook* review training will be documented in the PJ's medical training record. **Note:** ARC unit medical directors will not provide routine in-garrison medical care or oversee medical training unless the medical director is in an appropriate official duty status and working under the authority of the MTF with activation of clinical privileges.

4.5.2. PJs will maintain NREMT certification and training. **(T-1)** PJ continuing medical education and training report requirements in AFMAN 10-3500V1 will be accomplished quarterly.

4.5.3. NREMT recertification is required every two years. Completion of the SOCMID recertification or bridge course is the recommended for AFSPECWAR PJs maintaining Special Operations Advanced Tactical Paramedic certification.

4.5.4. When performing biennial patient contact/trauma sustainment rotations, PJs will review and follow all local protocol instructions. PJs will not perform clinical duties outside their scope of practice. **(T-1)**

4.5.5. Medical Skills Certifications Requirements. PJs will perform medical exercise and Medical Skills Certification requirements through sustainment training at the unit level. **(T-1)** AFMAN 10-3500V1 provides guidance on medical exercises and medical skills certifications requirements for PJs and supervising medical directors.

Chapter 5

AFSPECWAR MEDICAL EQUIPMENT AND SUPPLIES

5.1. Budgeting & Funding for Medical Supplies.

5.1.1. The Squadron Medical Logistics function will maintain a bench stock of medical supplies overseen by the Medical Logistician assigned to that unit. **(T-3)** See DAFI 41-230, *Medical Logistics Support*, for additional guidance.

5.1.2. For AFSPECWAR units, budgeting for medical supplies falls into two categories: WRM allowance standard items and non-AS materiel. WRM AS items are funded through the Air Force Working Capital Fund. WRM AS items are intended for deployment and use during initial stages of an operation until sustainable logistical lines are established or otherwise related activities. Consumed WRM AS items will be restocked and funded through unit O&M funding. **(T-2)**. If used to support a JTF exercise or deployment, they may be billed against the deployment fund cite. Refer to DAFI 65-601, Volume 1, *Budget Guidance & Procedures*. Non-AS materiel refers to medical materiel for all other routine use or training and is funded through unit O&M funding.

5.1.3. This bench stock may be procured and maintained with operations and maintenance funds and ordered through DMLSS. Suitable substitutions are acceptable when primary items are not available and as the PJ Med Handbook changes, but the AS has not been updated.

5.1.4. All IOS programs are line-funded and the responsibility of the line program management office or line commander for personnel, resources, and sustainment. DAF line (O&M) funding will be used for all IOS activities.

5.1.5. An MOA with the host MTF will include procedures for purchasing medical supplies through DMLSS, to include transfer of line-side funding for DMLSS purchases.

5.2. Medical War Reserve Materiel (WRM).

5.2.1. Medical WRM is owned by the Air Force Working Capital Fund/Medical Dental Division (AFWCF/MDD) operated, maintained, and sustained by AFMEDCOM by way of the host MTF medical logistics account. The medical WRM program prepositions or co-locates assets to a support unit that would use the materiel. This ensures assets are available when and where the medical mission requires, as reflected in applicable war plans. AFSPECWAR units that are not MTF based in nature do not have oversight of the AFWCF/MDD. Therefore, AFSPECWAR non-MTF units will not manage WRM until an MOA/MOU is established for exercise purposes or the WRM is sold to the base deployment account for an official contingency tasking.

5.2.2. Guidance on responsibility for WRM ownership and management duties of the Stock Record Accountant (previously known as the Accountable Base Medical Supply Officer) is provided in DAFI 41-230, paragraph 8.14.1.

5.2.3. Each WRM assemblage has the ability to support an austere location with no established medical supply chain for a minimum of 30 days, respective to the mission. If a location with an established medical supply chain has operational requirements that exceed the capabilities and time parameters of the primary deployed supply chain, a WRM asset can be authorized/justified for activation when approved by the MAJCOM MEFPAK. Special Operations Forces Medical and Special Tactics (81-series) unity type code identifiers can be used interchangeably in order to accommodate the AFSPECWAR deployed mission. The deploying unit embedded medical logistics function should coordinate with the PJ MMM to facilitate MEFPAK approval.

5.2.3.1. Medical WRM unit type code activations will be coordinated through the PJ MMM and asset deployment approved by the MAJCOM MEFPAK.

5.2.3.2. If a medical WRM unit type code is activated to support a joint task force exercise or deployment, it may be billed against the deployment fund site. Refer to DAFI 65-601V1. Non-allowance standard materiel refers to medical materiel for all other routine use or training and is funded through unit operations and maintenance funding.

5.2.3.3. Medical WRM utilized for deployment purposes is considered expended as theater medical supplies, therefore is transferred to the deployed area of responsibility.

5.2.3.3.1. The embedded deployed medical logistician will not report usage of the supplies or equipment back to the home-station host MTF Medical Logistics unless there is clear evidence that the WRM was not properly maintained prior to activation.

5.2.3.3.2. Evidence to WRM deficiencies include, but are not limited to, excess of expired consumable supplies and pharmaceuticals, inoperable medical equipment, reported adverse effects experienced by patients, or evidence that the asset was not temperature controlled (e.g., products are melted, dry-rotted, extreme discoloration).

5.2.3.4. Medical equipment deployed to an area of responsibility is gained to the deployed unit's medical equipment account. The historical maintenance records and equipment inventory documents will be printed at the time of transfer. The historical maintenance record will be provided to the deployed location supporting MTF Medical Logistics function for continued accountability and preventative maintenance/repair by the deployed biomedical equipment technician activity. A copy of the equipment operator and technical manuals are included with the historical maintenance record for the deployed location.

5.2.3.5. Contingency medical equipment deemed unserviceable/irreparable will be turned into the host MTF Medical Logistics for proper disposal at the Defense Logistics Agency-Disposition Services locations.

5.2.4. This section specifies duties when WRM is under the control of AFSPECWAR units under an MOA established for exercise purposes.

5.2.4.1. The Squadron commander will appoint in writing a Squadron WRM Project Officer who will act as the focal point for issues involving the management and maintenance of the loaned WRM assets under an MOA/MOU.

5.2.4.2. The appointed unit WRM Project Officer will:

5.2.4.2.1. Coordinate with the host MTF to establish MOAs concerning storage and maintenance of WRM and will ensure budgetary requirements to replace items used or damaged while in the unit's possession. **(T-2)**

5.2.4.2.2. Maintain accurate accounting of all WRM supplies and equipment and provide inventory status to host base medical logistics when changes occur. **(T-2)** For specific reporting instructions, refer to DAFI 41-230 and AFI 10-201, *Force Readiness Reporting*.

5.2.4.2.3. Inventory and inspect the WRM asset(s) after each exercise, prior to returning to the host MTF. **(T-2)** Provide completed inventory documents reflecting the current quantities to the host MTF WRM program manager upon asset return. **(T-2)**

5.3. Deviations and Waivers.

5.3.1. In cases of extreme operational need or patient safety, temporary deviations from approved supplies, equipment and medications not specified within the WRM AS, non-WRM approved kits, and the *PJ Med Handbook* may be authorized by the unit medical director.

5.3.2. Units desiring permanent deviations will submit requests through the appropriate command channels to the PJ MD and PJ MPM. **(T-1)** Requests should contain the following information in narrative format:

5.3.2.1. Deviation from procedure or protocol.

5.3.2.2. Circumstances that necessitate the requirement.

5.3.2.3. Impact of denial.

5.3.2.4. Inclusive dates of the request.

5.3.2.5. Specific location the waiver is to be granted.

5.3.2.6. Unit/individuals requiring the waiver.

5.3.3. The PJ MD will present all new and existing waivers and deviations during the next PJ MOAB meeting for consideration to revoke, renew, or adopt as standard practice. **(T-1)**

5.3.4. Deviations occurring during mission execution should be reported to the PJ MD within 24 hours if feasible with a follow-on written waiver request submitted as soon as practical.

5.4. Funding for Supplies. DAF IOS programs are line-funded and the responsibility of the line program management office or line commander for personnel, resources, and sustainment. DAF line (O&M) funding will be used for all IOS activities.

Chapter 6

PHARMACEUTICAL CONTROL AND STORAGE

6.1. Specified Tasks. The following guidance ensures the proper safeguards, accounting, and control measures of all pharmaceuticals maintained by the AFSPECWAR squadrons or units. Guidance on controlled medical items inventory is provided in DAFI 41-230.

6.1.1. Squadron Commanders will:

6.1.1.1. Establish a support agreement with host MTF to ensure adequate support in the ordering of medications required for mission support (may be delegated to IOS medical personnel within unit). **(T-1)**

6.1.1.2. Appoint in writing:

6.1.1.2.1. A primary and alternate controlled medical item custodian. **Note:** The controlled medical item custodian should be a Medical Logistician (4A1X1) or IDMT (4N0X1C). If neither is an option, consider other medical staff members or logistics career fields before other AFSCs or civilian personnel. (e.g., 2S0X1, 4N0X1).

6.1.1.2.2. An accountable medical supply officer.

6.1.1.2.3. A controlled substance destruction officer. **(T-1)** A sample template is provided in **Attachment 4**.

6.1.1.2.4. Reference DoDI 5000.64_DAFI 23-111, *Accountability and Management of DoD Equipment and Other Accountable Property,* for appropriate personnel who can act in this capacity.

6.1.1.2.5. All appointment letters will be maintained with the medical logistics function. See **Attachment 4** for a sample template.

6.2. Storage, Management, and Documentation. The Squadron Medical Materiel Manager will ensure all medical materiel is stored, managed, and documented.

6.2.1. Materiel Management. Refer to DAFI 41-230 for additional guidance. This includes, but is not limited to:

6.2.1.1. Provide adequate storage space that meets environmental and security requirements. **Note:** If adequate storage is unavailable, the commander must be notified immediately to identify a suitable solution.

6.2.1.2. Secure all warehouse doors and storage areas with locks to prevent theft and pilferage.

6.2.1.3. Establish procedures for stock rotation and timely replacement.

6.2.1.4. Ensure proper handling and disposal of expired medications.

6.2.2. Documentation. Maintain and dispose of medical materiel records. Refer to AFI 33-322 and the AF RDS located in the Air Force Information Management System. **Note:** If there are guidance conflicts, utilize the longer retention period.

6.3. Storage, Management, and Documentation of Controlled Medical Items in Support of AFSPECWAR Missions and Medical Coverage.

6.3.1. The controlled medical item custodian will follow processes outlined in DAFI 41-230 for controlled substances to include proper ordering, issuance, storage, inventory, and record-keeping. **(T-0)** Refer to DAFI 41-230 for more guidance on controlled medical items storage procedures.

6.3.2. Only the appointed controlled medical item custodian and the accountable medical supply officer will know the combination to vault and caged storage areas. The Standard Form (SF) 700, *Security Container Information*, will be utilized to maintain a record for the combination. The SF 700 will be classified at the same level as the highest classification of the material authorized for storage in the container and stored in a vault or safe of equivalent classification IAW DoD Manual (DoDM) 5200.01, Volume 3*, DoD Information Security Program: Protection of Classified Information*. **(T-1)**

6.3.3. The Squadron medical logistics function will establish operating instructions to follow during an alert response, or in case of an emergency when additional controlled substances are required to support a mission. **(T-3)**

6.3.4. Controlled pharmaceuticals within activated WRM assets will be controlled in the same manner as in-garrison assets. At a minimum, items will be secured in locked rooms and containers. **(T-2)**

6.3.4.1. During temporary duty (TDY) or deployments, controlled substances will be safeguarded using the following procedures:

6.3.4.1.1. Should be stored double-locked in a secure area with 24-hour access (alert facility/rescue coordination center, MTF, security forces, etc.). **(T-3)**

6.3.4.1.2. In the absence of an approved safe, vault, or secure storage area, the controlled substances will be maintained on the deployed medical logistician or other identified medical staff. **(T-3)**

6.3.4.2. Controlled substances may be stored on board mission aircraft if they are in a high valued item bin or placed in a standard weapons storage container when the container is used solely for medication storage. Storage containers must be secured to a floor tie down ring, and the container hasp must be secured with a combination lock. **Note:** When required, the combination should only be given to the aircraft commander. **(T-2)**

6.3.4.2.1. A daily inspection of the controlled substances stored on any aircraft will be completed by the team leader or team member who signed for the controlled substances being stored. Loss or theft at a TDY or deployment location will immediately initiate investigation and must be reported to the commander of the storage facility and to the home station squadron commander. **(T-3)**

6.3.4.2.2. Access to the controlled substance bin must be authorized by the team leader or team member who signed for the controlled substances being stored. **(T-2)**

6.4. Issuance of Controlled Medical Items in Support of AFSPECWAR Missions and Medical Coverage.

6.4.1. If the use of DMLSS or a biometric locker system or other approved storage system is unavailable, the controlled medical item custodian is authorized to build controlled medication kits for authorized personnel as required for mission execution. Record the individual issue on the items AF Form 579, *Controlled Substances Register,* and provide the member with a copy of a DAF Form 1297, *Temporary Issue Receipt*. The option to utilize a sign-out/sign-in logbook for pre-built controlled item kits is also available in lieu of the DA Form 1297.

6.4.2. Personnel who sign out controlled substances will:

6.4.2.1. Sign out controlled substances for requisite alert status, training, TDY, deployment, or mission. **(T-0)**

6.4.2.2. Sign in controlled substances at first available opportunity upon return from TDY, alert status, deployment, or mission. **(T-0)**

6.4.2.3. Be solely responsible and accountable for the security and safeguard of the signed out controlled substances. Signed out narcotics will not be handed off to teammates or other personnel for turn-in unless extenuating circumstances apply. **(T-0)** Extenuating circumstances will be considered for approval on a case-by-case basis by the unit medical director prior to an event in which controlled substances are required. In the event of an unforeseen emergency (e.g., a team member becoming a casualty) that prevents the assigned member from turning in controlled substances, another qualified team member may take possession. This action must be documented at the earliest opportunity and reported to the unit medical director upon mission conclusion.

6.4.2.4. Inspect controlled substances for adequate quality and quantity whenever they are signed out or prior to assumption of alert duty. Report discrepancies to the medical logistics function immediately. **(T-2)**

6.4.2.5. Report type and quantity to the controlled medical item custodian within 12 hours when controlled substances are used. In the event of ongoing operations, every effort will be made to report use within 12 hours of mission completion. Prescribed controlled substances must be documented in either SF 600, *Chronological Record of Medical Care*, Battlefield Assisted Trauma Distribution Observation Kit or the Consolidated Mission Report and must be reported to the unit's medical logistician upon return. **(T-1)**

6.4.2.6. Inspect controlled substances for adequate quality and quantity prior to turn in. If discrepancies are noted, report them to the controlled medical item custodian immediately. **(T-3)**

6.5. Transportation of Controlled Medical Items.

6.5.1. Include a statement authorizing personnel to carry professional gear such as medical kits containing various medications, including controlled substances, in the remarks section of administrative orders.

6.5.2. It is best to list an inventory of specific medications actually transported. If this is not possible, list "Narcotics Courier" on the orders of all personnel potentially able to carry controlled substances.

6.6. Loss Reporting.

6.6.1. When a loss or theft of controlled substances occurs, the controlled medical item custodian will immediately notify the squadron commander. The squadron commander will initiate an investigation. If the initial investigation indicates a probable theft or diversion, as opposed to an administrative error, the commander will contact the Office of Special Investigations (OSI).

6.6.2. Prepare Drug Enforcement Agency (DEA) Form 106, *Report of Loss or Theft of Controlled Drugs*. All loss or theft of controlled substances needs to be reported to the host MTF through the Medical Logistics Flight Commander who holds the DEA certificate. In some situations, the pharmacist may hold the DEA certificate. Whichever entity holds the DEA certificate will submit the DEA Form 106 to the nearest DEA regional office. **(T-0)** DEA Form 106 is available from DEA regional offices.

6.7. Controlled Medical Items Commercial Credit Return and Destruction.

6.7.1. Process commercial credit return IAW DAFI 41-230, paragraph 5.11 for expired controlled pharmaceuticals. **(T-3)**

6.7.2. Broken vials/containers requiring destruction or controlled medications drawn, but not used, and require waste/destruction will be recorded on an AF Form 579. The AF Form 579 will be accompanied by a memorandum for record citing the reason for destruction or waste. Both the responsible member and the supporting provider will sign the AF Form 579 and the memorandum.

6.7.3. In the contingency environment, expired turn ins and destructions will be processed by the expeditionary medical logistics function at the host Expeditionary MDG or equivalent. AFSPECWAR medical logisticians unable to generate DMLSS-based return documents will manually prepare the Expeditionary MDG, or equivalent, departmental supply chain applicable form for controlled item turn in. (i.e., DA Form 3161, *Request for Issue or Turn-In;* AF Form 579). **(T-1)**

6.7.4. Maintain all controlled substance source documentation (delivery receipts, AF Form 579, DA Form 3161, etc.) for a period of three years.

CASE A. CUNNINGHAM,
Lt General, USAF
Deputy Chief of Staff, Operations

Attachment 1

GLOSSARY OF REFERENCES AND SUPPORTING INFORMATION

References

Title 10 United States Code Section 9036, *Surgeon General: Appointments; Duties*

Title 21, Code of Federal Regulations, Parts 1301.71-1301.76, *Security Requirements*

PL 115-232, *National Defense Authorization Act for Fiscal Year 2020*, Section 712

PL 116-92, *National Defense Authorization Act for Fiscal Year 2020*, Section 712(a)

DoDI 5000.64_DAFI 23-111, *Accountability and Management of DoD Equipment and Other Accountable Property*, 6 December 2021

DoDI 6025.13, *Medical Quality Assurance and Clinical Quality Management in the Military Health System*, 26 July 2023

DoD Manual 5200.01, Volume 3*, DoD Information Security Program: Protection of Classified Information*, 24 February 2019

DHA-PM 6025.13, *Clinical Quality Management in the Military Health System,* 19 August 2019

DAFPD 10-35, *Air Force Special Warfare,* 5 January 2021

HAFMD 1-48, *The Air Force Surgeon General*, 21 February 2023

HAFMD 1-54, *Deputy Chief of Staff, Operations*, 8 September 2015

DAFI 41-106, *Medical Readiness Program*, 29 July 2020

DAFI 41-230, *Medical Logistics Support*, 24 July 2025

DAFI 65-601, Volume 1, *Budget Guidance & Procedures*, 6 May 2026

DAFI 90-302, *The Inspection System of the Department of the Air Force*, 15 March 2023

AFI 10-201, *Force Readiness Reporting,* 5 June 2024

AFI 10-3503, *Personnel Parachute Program*, 23 September 2020

AFI 10-403, *Deployment Planning and Execution*, 17 December 2025

AFI 25-201, *Intra-Service, Intra-Agency, and Inter-Agency Support Agreements Procedures*, 18 October 2013

AFI 33-322, *Records Management and Information Governance Program*, 23 March 2020

AFI 44-103, *The Air Force Independent Duty Medical Technician Program*, 30 August 2018

AFI 44-108, *Infection Prevention and Control Program*, 5 June 2019

AFI 44-119, *Medical Quality Operations*, 16 August 2011

DAFMAN 10-211, *Integrated Operational Support (IOS)*, 22 August 2025

DAFMAN 48-105, *Public Health Surveillance*, 11 October 2023

DAFMAN 90-161, *Publishing Processes and Procedures*, 18 October 2023

AFMAN 10-3500, Volume 1, *AFSPECWAR Training Program,* 3 June 2025

AFMAN 10-3511, *Pararescue and Combat Rescue Officer Operations*, 2 December 2020

AFMAN 24-604, *Preparing Hazardous Materials for Military Air Shipments*, 6 June 2025

AFMAN 48-149, *Flight and Operational Medicine Program (FOMP)*, 13 October 2020

Department of the Air Force *Enlisted Classification Directory (DAFECD)*

Department of the Air Force *Officer Classification Directory (DAFOCD)*

USAF *Pararescue Medical Operations Handbook*, current edition

Prescribed Forms

None

Adopted Forms

AF Form 579, *Controlled Substances Register*

DAF Form 847*, Recommendation for Change of Product*

DAF Form 1297*, Temporary Issue Receipt*

DD 1380, *Tactical Combat Casualty Care (TCCC) Card*

DA Form 3161, *Request for Issue or Turn-In*

DA Form 4700, *Medical Record – Supplemental Medical Data*

DEA Form 106*, Report of Loss or Theft of Controlled Drugs*

SF 600*, Chronological Record of Medical Care*

SF 700, *Security Container Information*

Abbreviations and Acronyms

AAR—After Action Report

AF—Air Force

AFI—Air Force Instruction

AFMAN—Air Force Manual

AFMEDCOM—Air Force Medical Command

AFRC—Air Force Reserve Command

AFSC—Air Force Specialty Code

AFSOEMS—Air Force State Office of Emergency Medical Services

AFSPECWAR—Air Force Special Warfare

ANG—Air National Guard

ARC—Air Reserve Component

AS—Allowance Standards

CFM—Career Field Manager

DA—Department of the Army

DAF—Department of the Air Force

DAFECD—Department of the Air Force Enlisted Classification Directory

DAFI—Department of the Air Force Instruction

DAFMAN—Department of the Air Force Manual

DAFOCD—Department of the Air Force Officer Classification Directory

DAFPD—Department of Defense Policy Directive

DD—Department of Defense (as used on forms)

DEA—Drug Enforcement Agency

DHA—Defense Health Agency

DMLSS—Defense Medical Logistics Support System

DoD—Department of Defense

DoDI—Department of Defense Instruction

DoDM—Department of Defense Manual

DoW—Department of War

EHR—Electronic Health Record

EMS—Emergency Medical Services

HAFMD—Headquarters Air Force Mission Directive

HPAB—Human Performance Advisory Board

HPO—Human Performance Optimization

IAW—In Accordance With

IDMT—Independent Duty Medical Technician

IOS—Integrated Operational Support

JTS—Joint Trauma System

MAJCOM—Major Command

MD—Medical Director

MDG—Medical Group

MEFPAK—Manpower and Equipment Force Packaging

MFM—MAJCOM Functional Manager

MICT—Management Internal Control Toolset

MMM—Medical Material Manager

MOA—Memorandum of Agreement

MOAB—Medical Operations Advisory Board

MOU—Memorandum of Understanding

MRDSS—Medical Readiness Decision Support System

MTF—Medical Treatment Facility

NREMT—National Registry of Emergency Medical Technicians

OCS—Operational Clinical Services

OPR—Office of Primary Responsibility

PJ—Pararescue or Pararescueman

PJ MD—Air Force Pararescue Medical Director

PJ MMM—Air Force Pararescue Medical Material Manager

PJ MPM—Air Force Pararescue Medical Programs Manager

PJ Med Handbook—USAF Pararescue Medical Operations Handbook

PL—Public Law

QA—Quality Assurance

SERE—Survival, Evasion, Resistance and Escape

SF—Standard Form

SOCMID—Special Operations Center for Medical Integration and Development

TCCC—Tactical Combat Casualty Care

TDY—Temporary Duty

USAF—United States Air Force

USSOCOM—United States Special Operations Command

WRM—War Reserve Materiel

Office Symbols

ACC/SGP—Air Combat Command Chief of Aerospace Medicine

AETC/A3/6—Air Education and Training Command, Operations and Communications

AETC/SGP—Air Education and Training Command, Chief of Aerospace Medicine

AF/A34S—Air Force Special Warfare Division

AF/A34SG—Air Force Special Warfare Medical Director

AF/HC—Air Force Chief of Chaplains

AF/SG—Air Force Surgeon General

AF/SG3P—Air Force Chief of Aerospace Medicine

AFMEDCOM/SG4T—AFMEDCOM Medical Logistics Training Directorate

AFRC/SGP—Air Force Reserve Command Chief of Aerospace Medicine

AFRC/A3J—Air Force Reserve Command, Directorate of Plans, and Requirements

AFSOC/SGP—Air Force Special Operations Command Chief of Aerospace Medicine

AFWCF/MDD—Air Force Working Capital Fund Medical Dental Division

ANG/SGP—Air National Guard Chief of Aerospace Medicine

MAJCOM/A3—Major Command Operations Division

MAJCOM/SG—Major Command Surgeon General

MAJCOM/SGX—Major Command Chief of Medical Readiness

MDG/SGH—Medical Group Chief of Staff

MDG/SGP—Medical Group Chief of Aerospace Medicine

OSI—Office of Special Investigations

PACAF/SGP—Pacific Air Forces Chief of Aerospace Medicine

SQ/CC—Squadron Commander

USAFE/SGP—United States Air Forces in Europe Chief of Aerospace Medicine

Terms

Battlefield Assisted Trauma Distribution Observation Kit—A mobile, real-time health monitoring and documentation application developed by the United States Air Force Research Laboratory. It is designed to assist medics and other medical personnel in the field by providing a digital platform for tracking, recording, and managing patient care during combat or other operational scenarios.

Controlled Medical Items—A specific type of medical material that requires careful tracking and management due to its potential for misuse or diversion.

Controlled Medication Letter—A written form or document used to communicate information about medication use, particularly when dealing with medications that may affect flight duty or operational performance.

Controlled Substance—Drugs so designated by the Attorney General because of demonstrated or potential abuse.

Office of Primary Responsibility—The office or unit directly responsible for completing a specific task, process, or product.

Pararescue Medical Scope of Practice—The range of procedures, actions, and processes Pararescue personnel are permitted to perform based on their education, training, experience, and license.

Risk Management—A comprehensive approach that includes clinical and administrative activities, processes, and policies to identify, monitor, assess, mitigate, and prevent risks to the health care organization, patients, and staff. A process that assists organizations and individuals in making informed risk decisions to reduce or offset risk, thereby increasing operational effectiveness and the probability of mission success.

Squadron Medical Element—A line funded medical support team package, comprised of a flight surgeon and one or two medical technicians, assigned or attached to a qualifying aviation unit.

Tactical Combat Casualty Care (TCCC)—A comprehensive training program designed to equip Airmen with the skills to provide life-saving trauma care in a pre-hospital environment, particularly on the battlefield.

War Reserve Materiel (WRM)—A strategically managed stockpile of essential equipment, supplies, and munitions that the Air Force maintains to support operations during wartime and other contingencies.

Attachment 2

HUMAN PERFORMANCE ADVISORY BOARD (HPAB) EXECUTIVE COUNCIL

A2.1. HPAB Executive Council Members. See **Table A2.1**.

A2.1.1. Review and validate the HPAB charter biennially. **(T-1)**

A2.1.2. Provide advice to the AF/A34S Director on AFSPECWAR IOS HPO, including HPO policies, procedures, equipment and technology standardization issues IAW the HPAB charter.

A2.1.3. At a minimum, convene annually in person or within 48 hours for urgent issues.

A2.1.4. The Chair may call executive sessions, which include the Board Chair, voting members, and HPO advisors.

A2.1.5. Decisions require a simple quorum vote; the Board Chair will break any ties.

Table A2.1. HPAB Executive Council Members.

Roles	Members
Board Chair	HPO Director
Voting Members[1]	One AFSPECWAR representative from the 3-Ltr staff of the following MAJCOMs: 1. Air Combat Command 2. Air Education and Training Command 3. Air Force Special Operations Command 4. United States Air Forces in Europe 5. Pacific Air Forces 6. AFRC 7. ANG
Non-Voting Advisors	HPO personnel, as selected by each voting member
Notes: 1. All representatives must be from the 1Z or 19Z career fields. **(T-1)** 2. A quorum for any vote requires the presence of 75% of the voting members. 3. For HPO-wide issues, a 70% majority of the quorum is required to forward a recommendation to the AF/A34S. 4. Proxies for voting members are permitted but must be designated in writing, with their specific voting rights clearly specified. **(T-1)**	

Attachment 3

PJ MEDICAL OPERATIONS ADVISORY BOARD (MOAB) EXECUTIVE COUNCIL

A3.1. PJ MOAB Executive Council Members. See **Table A3.1**.

A3.1.1. Review and validate the PJ MOAB charter biennially. **(T-1)**

A3.1.2. Evaluate waivers and deviations during meetings for consideration to revoke, renew, or adopt as standard practice.

A3.1.3. Provide advice on PJ medical operations, including policy, training, and materiel IAW the MOAB charter.

A3.1.4. At a minimum, convene annually in person or within 48 hours for urgent issues. **(T-1)**

A3.1.5. The Chair may call executive sessions, which include the Board Chair, voting members, and PJ MPM.

A3.1.6. Decisions require a simple quorum vote; the Board Chair will break any ties.

Table A3.1. PJ MOAB Executive Council Members.

Roles	Members
Board Chair	PJ MD
Voting Members	1. Air Force Chief of Aerospace Medicine (AF/SG3P) 2. Pararescue Career Field Manager (PJ CFM) 3. Air Combat Command Chief of Aerospace Medicine (ACC/SGP) 4. Air Combat Command PJ MFM 5. Air Education and Training Command Chief of Aerospace Medicine (AETC/SGP) 6. Air Education and Training Command PJ MFM 7. Air Force Special Operations Command Chief of Aerospace Medicine (AFSOC/SGP) 8. Air Force Special Operations Command PJ MFM 9. Pacific Air Forces Chief of Aerospace Medicine (PACAF/SGP) 10. Pacific Air Forces PJ MFM 11. United States Air Forces in Europe Chief of Aerospace Medicine (USAFE/SGP) 12. United States Air Forces in Europe PJ MFM 13. Air Force Reserve Command Chief of Aerospace Medicine (AFRC/SGP) 14. AFRC PJ MFM 15. Air National Guard Chief of Aerospace Medicine (ANG/SGP) 16. ANG PJ MFM
Non-Voting Advisors	1. AF/A34SG 2. PJ MPM

Notes:
1. A quorum requires at least 12 members, comprising at least six physicians and six PJ representatives.
2. Proxies for voting members must be designated in writing with voting rights clearly specified. **(T-1)**

Attachment 4

SAMPLE APPOINTMENT LETTER TEMPLATE

Table A4.1. Sample Appointment Letter Template.

DD MMM YYYY

MEMORANDUM FOR RECORD

SUBJECT: [Insert Position Title] Appointment Letter

1. The purpose of this memorandum is to provide appointment notification for the role of [insert position here].

2. In accordance with AFMAN 10-3509, [INSERT RANK & NAME] is officially appointed as the [insert unit here] [insert position here].

3. Questions regarding this memorandum may be directed to [insert SQ/CC name, at [insert email] or [contact number].

NAME
Signature Block